

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°138/MVE-BDBV/09/29/2026



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HIGHLIGHTS (last 24 hours)

During the day of September 29, 2026, 53 new confirmed cases of Ebola Virus Disease (EVD), including 22

Community deaths have been reported. The confirmed cases were distributed across the provinces of Ituri (32 cases), North Kivu (17 cases), and Haut-Uélé (4 cases). In addition, 6 deaths were recorded in Ebola Treatment Centers (ETCs).

A total of 328 new samples were analyzed; 53 of these samples were found to be positive for the Bundibugyo virus, representing a positivity rate of 16.2%.

No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 8,224	ACCUMULATIVE DEATH CONFIRMED 3,982	LETHALITY 48.4%	PATIENTS IN ISOLATION/CTE 851	ACCUMULATIVE HEALED 2,121	TRACKING RATES CONTACTS (Of the Day) 78.7%
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Affected provinces and affected health zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



63 out of 167 health zones affected (37.7%)

Summary of key points from the last 24 hours

Fifty-three (53) new confirmed cases of Ebola Virus Disease (EVD), including **22 community deaths**, have been recorded in the last 24 hours. The provinces reporting these cases are Ituri (32 cases), North Kivu (17 cases) and Haut-Uélé (4 cases).

In addition, **6 deaths were recorded in the CTEs** (4 in Ituri, 1 in North Kivu and 1 in Haut-Uélé), bringing the total number of confirmed deaths for the period under consideration to 28.

However, 16 patients (**12 in Ituri, 3 in North Kivu and 1 in Haut-Uélé**) were declared cured after obtaining two negative results for Ebola virus disease.

As of September 29, 2026, the DRC has recorded **8,224 confirmed cases and 3,982 deaths**. (Case fatality rate: **48.4 %**). Ituri remains the epicenter of the epidemic with **76.0%** of cumulative confirmed cases reported and **60.4%** of new confirmed cases. In contrast, North Kivu province has a case fatality rate higher than the national average, at **59.3%**.

In total, **63 out of 167 health zones (37.7%)**, spread across **seven provinces**, have been affected since the start of the epidemic.

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 29, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	32	6250	2,885	46.2%	28/36 (77.8%)
North Kivu	17	1570	931	59.3%	16/34 (47.1%)
Haut-Uélé	4	344	144	41.9%	7/13 (53.8%)
Tshopo	0	45	16	35.6%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	10	4	40.0%	3/11 (27.3%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
Total	53	8,224	3,982	48.4%	63/167 (37.7%)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 29, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6250	2885	46.2%	32	13	4	17
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	202	46	22.8%				0
Boga	2	2	100.0%				0
Bunia	1728	561	32.5%	14	7		7
Damascus	40	9	22.5%				0
Drodro	15	7	46.7%				0
Fataki	82	31	37.8%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	35	12	34.3%				0
Komanda	169	102	60.4%	1		1	1
Lita	284	143	50.4%				0
Logo	13	5	38.5%				0
Lolwa	34	9	26.5%	1		1	1
Mahagi	4	1	25.0%				0
Mambasa	29	11	37.9%				0
Mandima	137	75	54.7%	6	4		4
Mangala	351	154	43.9%	1	1		1
Mongbwalu	701	331	47.2%	1		1	1
Nia-Nia	298	162	54.4%	3	1		1
Nizi	778	279	35.9%	3			0
Nyankunde	129	37	28.7%	1			0
Rimba	12	4	33.3%				0
Rwampara	1083	369	34.1%	1		1	1

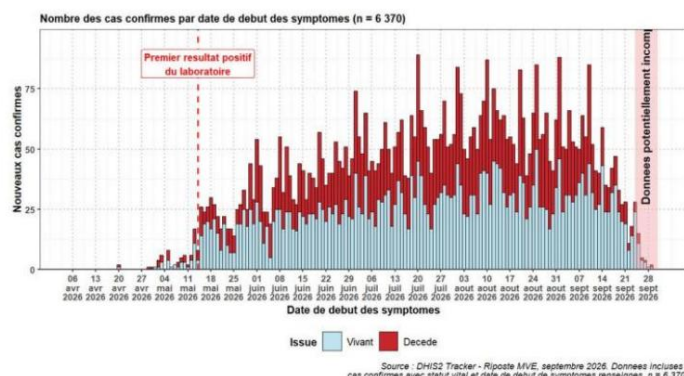
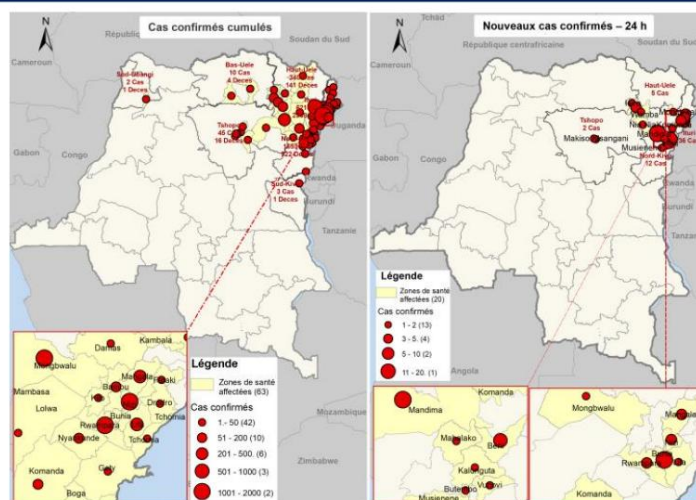
Tchomia	76	25	32.9%				0
To ventilate *	N / A	493	N / A				0
North Kivu	1570	931	59.3%	17	8	1	9
Blessed	348	229	65.8%	5	1	0	1
Good	19	7	36.8%	1			0
Butembo	263	170	64.6%	5	3	0	3
Goma	1	0	0.0%				0
Kalunguta	54	18	33.3%				0
Katwa	562	357	63.5%	3	2	0	2
Kayna	1	0	0.0%				0
Kyondo	33	20	60.6%				0
Lubero	21	11	52.4%				0
Mabalako	33	12	36.4%	1	1	0	1
Manguredjipa	13	5	38.5%				0
Masereka	29	9	31.0%	1	0	0	0
Musienene	124	67	54.0%	1	1	0	1
Mutwanga	12	2	16.7%				0
Oicha	26	14	53.8%				0
Vuhovi	31	10	32.3%	0	0	1	1
Haut-Uélé	344	144	41.9%	4	1	1	2
Boma Mangbetu	39	17	43.6%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	102	44	43.1%				0
Pawa	85	39	45.9%	2	0	1	1
Rungu	2	1	50.0%				0
Wamba	112	41	36.6%	2	1	0	1
Tshopo	45	16	35.6%	0	0	0	0
Bafwasende	7	1	14.3%				0
Kabondo	5	1	20.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	24	10	41.7%				0
Mangobo	6	3	50.0%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	10	4	40.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Ganga	3	2	66.7%				0
South Ubangi	2	1	50.0%	0	0	0	0
Bulu	2	1	50.0%				0
Total	8224	3982	48.4%	53	22	6	28

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for September 29, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	0	0	0	0	0	0	0	0	0
Bas Uélé	118	2	120	12	1	107	0	13	0
Ituri	1,132	67	1199	143	67	989	0	210	129
North Kivu	459	0	459	112	0	347	0	112	88
South Ubangi	42	0	42	0	0	42	0	0	0
Tshopo	149	2	151	10	2	139	0	12	1
Total General	1,900	71	1971	277	70	1,624	0	347	218

Figure 1. Evolution of cases by date of symptom onset over the last 21 days**Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.**

1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

During the course of the day on September 29, 2026, **1,971** alerts were raised, **1,971** (100,0 % were verified. All **334** (16.9%) alerts validated as suspected cases were investigated. Thus, 218 suspected cases were transferred to CT/CTE ([Table 3](#)).

Of the **30,751** contacts being monitored, **24,206** were seen in the last 24 hours, representing a follow-up rate of **78.7 %**, lower than the previous day's rate (82.0%) and the required threshold (95.0%). This rate was 93.9% (92/98) in South Ubangi, 85.3% (12,248/14,363) in Ituri, 87.0% (161/185) in Bas-Uélé, 83.7% (638/762) in Tshopo, 71.4% (10,260/14,367) in North Kivu, and 82.7% in Haut-Uélé (807/976).

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Five (5) alerts were notified to the PoE/PoC during the day, including one (1) in Ituri, two (2) in North Kivu, and two (2) in Tshopo (PoC PK 15, Buta road). In Ituri, the alert from the Mabakese PoC (Mambasa Health Zone), concerning a traveler from Musienene, was validated, and the suspect was transferred to the Mambasa CTE (transfer rate of 100%). In North Kivu, the two alerts from the Karuruma PoC were invalidated after investigation.

Table 4. Status of key PoE/PoC indicators by province, as of September 29, 2026

Indicators	Ituri	North Kivu	Tshopo	Overall
Report completeness	96.7% (58/60)	95.2% (20/21)	100.0% (18/18)	97.0% (96/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	138,080	94,428	19,533	252,041
% of travelers screened	96.6%	98.2%	97.9%	97.3%
% of travelers who washed their hands	96.3%	98.2%	98.1%	97.2%
% of travelers made aware	99.5%	98.2%	100.0%	99.1%
Number of alerts notified	1	2	2	5
Number of validated live alerts (suspected cases)	1	0	0	1
Number of bodies recovered today	0	0	0	0
% of suspected cases transferred to CTE/CT	100.0%	N / A	N / A	100.0%
% of lifeless bodies swabbed/secured	N / A	N / A	N / A	N / A
% of suspected cases investigated within 4 hours	100.0%	N / A	N / A	100.0%

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 32 new positive results** (19 living and 13 dead) out of 175 samples received and analyzed (**positivity: 18.3%**).
- **North Kivu: 17 new positive results** (9 living and 8 dead) out of 124 samples received and analyzed (**positivity: 13.7%**).
- **Haut-Uélé: 4 new positive results** (3 living and 1 death) out of 21 samples received and analyzed (**positivity: 19.0%**).
- **Tshopo: 6 samples received and analyzed** (6 live), all came back negative.
- **Bas Uélé: 2 samples received and analyzed** (2 live), all came back negative.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 19 out of 34 rings were opened (55.9%), 26 out of 37 households were decontaminated (70.3%), and 14 out of 17 health facilities were decontaminated (82.4%). Of 82 EDS alerts (including 5 postponements), 60 EDS were carried out (73.2%) and 55 bodies were swabbed.
- **In North Kivu,** 12 out of 19 rings were opened (63.2%), 3 out of 21 households were decontaminated (14.3%), and 15 out of 17 health establishments were decontaminated (88.2%). In total, 209 health facilities were monitored, 516 service providers were briefed, and 61 blood and sanitation surveys were conducted, with 45 bodies swabs collected out of 47 alerts received.
- **Haut-Uélé :** 3 out of 3 rings were opened (100%), 21 out of 24 households were decontaminated (87.5%) and 7 out of 9 ESS (Emergency Situations) were decontaminated (77.8%). Of 5 EDS (Emergency Disposal System) alerts, 3 EDS were carried out and 3 bodies were swabbed; 2 failures were recorded in Boma Mangbetu due to community resistance

- **Tshopo** : 18 health establishments (HEs) were monitored in 6 health zones and 186 service providers were briefed; integrated training for 150 private HEs started in Makiso-Kisangani, and the households of cases 38 and 39, as well as the Kawaki clinic (case 44), were decontaminated. Cumulatively, 34 out of 34 HEs (100%) and 26 out of 31 households (83.9%) that hosted a confirmed case were decontaminated within 48 hours.
- **Bas-Uélé** : 10 out of 10 ESSs were followed in Viadana, Buta and Ganga and 34 providers briefed (100%); PCI/WASH training of 35 PPLs and 35 hygienists continues in Ganga (FHI 360). Of 2 EDS alerts, 1 body was swabed and one failure was recorded at Koteli.

2.4.2. Challenges

In **Ituri**, the opening of the rings remains insufficient (55.9%; 19/34), 15 bodies were not swabed due to a lack of supplies and teams in Mandima or following refusal by families, and no household or health establishment (HE) received kits. In **North Kivu**, household decontamination remains very low (14.3%; 3/21), no decontaminated household received a kit (0/3), the average infection prevention and control (IPC) score of the 7 HEs assessed is 36.6%, and the timeliness/completeness of IPC reports is 56.3% (9/16 health zones).

2.5. Holistic care

2.5.1. Main actions

- In **Ituri**, 83 new admissions were recorded compared to 72 discharges, including 12 recoveries. In total, 392 patients remained in isolation at the end of the day. The bed occupancy rate was **38.6%** ; 94.1% of patients received hot meals and children under LNPE/SLM were fully covered at the CTE (15/15), in the nursery (5/5) and in the community (47/47); 35 patients are in critical condition.
- In **North Kivu**, 68 new admissions were recorded, compared to 68 discharges (3 recoveries). A total of 365 patients remained hospitalized, including 223 in standardized facilities. The bed occupancy rate was **63.0%**.
- In **Tshopo**, one new admission was recorded. No discharges were reported. Thirteen patients remained in isolation at the end of the day, representing an occupancy rate of **41.9%**.
- In **Bas-Uélé**, 4 new admissions were recorded. No discharges were reported. Eleven patients remained in isolation at the end of the day.
- In **Haut-Uélé**, 7 new admissions (1 confirmed and 6 suspected) were recorded, compared to 13 discharges, including 1 recovery, 11 non-cases, and 1 confirmed death at the Pawa Ebola Treatment Center (ETC). A total of 70 patients (32 confirmed and 38 suspected) remained in isolation in ETCs, representing an occupancy rate of **50.7%** ; 74 patients, 148 caregivers, and 45 people with disabilities received hot meals.

2.5.2. Challenges

- In **Ituri**, some CTE/CT facilities have limited capacity and inadequate care is provided. food-related issues remain a concern.
- In **Bas-Uélé**, the lack of a standardized CTE and ambulance for the transfer of patients remains a major operational challenge.
- In **Tshopo**, the continued presence of confirmed cases in isolation necessitates the maintenance of infection prevention and control measures.

2.6. Vaccination

2.6.1. Main actions

- In **Ituri**, 129 people were vaccinated on September 29 with MSF support, including 68 women and 61 men, with no reported adverse events following immunization (AEFIs). The cumulative total has reached 731 people vaccinated, including 319 women and 412 men. Vaccination activities and research on the efficacy of the ERVEBO vaccine against BDBV-related Ebola virus disease are ongoing.
- In **Bas-Uélé**, the provincial total reached 1,022 people vaccinated (PPL and TPL) out of a target of 13,202 (7.7%), primarily in Buta (572; 20.0%) and Ganga (273; 20.5%). The deployment of compassionate vaccination teams is being prepared in Aketi and Viadana.

2.6.2. Challenges

In **Ituri**, the target for PPL/TPL remains to be redefined, which prevents the calculation of a reliable coverage rate.

In **Bas-Uélé**, the UCCs have not been put into operation, the validated micro-plan and vaccination funds are not available.

2.7. Continuity of Care

2.7.1. Main actions

- In **Ituri**, the pillar assessed the continuity of vaccination services in 14 affected health zones, with an emphasis on mobile units, in order to recover zero-dose and under-vaccinated children.
- In **Haut-Uélé**, the evaluation of the secondary hospital of Nebobongo (ZS of Pawa) reveals a decrease of approximately 50% in attendance and more than 75% in prenatal consultations, with 55% of services functional (major disruption); 2 PPLs are being followed at the CTE of Wamba.
- In **Tshopo**, the Horizon Medical Centre was evaluated (2 out of 11 staff vaccinated, i.e. 18.2%, and triage area not effective), the CM CELPA recovery plan was developed and the training of zonal focal points on continuity of services began.
- In **Bas-Uélé**, the assessment of the Ganga health zone (May to August 2026) shows a use of services between 35 and 39% for 60% expected and a bed occupancy rate of 30.2 to 39.4% for 70% expected; no triage area is functional and a PPL has been infected.

2.7.2. Challenges

In **Tshopo**, there has been a decrease in the use of services, particularly maternity and cesarean sections, in the assessed ESS; the temporary closure of the DHKis ESS remains proposed in the face of the multiplication of confirmed and suspected cases.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- In **Ituri**, 4,479 home visits reached 19,349 people; 276 educational talks brought together 3,535 people, 8 community debates reached 408 people, and 44 mass awareness campaigns reached 1,089 people. A total of 163 community alerts, including 67 deaths, were reported; 123 out of 135 feedback reports (91.1%) were received, and the 5 recorded incidents were resolved within 48 hours (9 out of 28 health zones completed).
- In **North Kivu**, 9,231 home visits reached 28,227 people and 39 educational talks reached 1,087 people; 259 out of 650 expected community alerts (39.8%) were notified and 19 out of 25 feedback requests (76.0%) were resolved within 48 hours. The CREC facilitated the listing of 93 contacts in Manguredjipa, 11 in Lubero, and 9 in Oicha.
- In **Tshopo**, 208 home visits reached 1,165 people, 4 educational talks reached 162 people and 12 mass awareness sessions reached 3,102 people; the 64 community alerts notified and the 2 feedbacks were dealt with, and 14 schools as well as 55 learners from ITM La Grâce à Dieu (Lubunga) were sensitized.
- In **Bas-Uélé**, 6 educational talks and 4 mass awareness campaigns reached 830 people in Viadana, Titule and Ganga; an interactive program on community radio Bima (Titule) responded to the rumor of September 28 about the vaccination of students and 16 leaders were sensitized in Poko.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- In **North Kivu**, 12 confirmed cases (100%) recorded the previous day and 246 confirmed cases under follow-up benefited from SMSPS interventions in the 6 health zones that reported. The teams assisted 34 households of separated and orphaned children (100%), announced 98 results, including 12 positive ones, and reintegrated 8 recovered children.
- In **Ituri**, 19 of the 36 most recent confirmed cases, 80 confirmed cases currently being monitored out of 148 (54.1%), 36 new suspected cases out of 83 (43.4%), and 113 suspected cases currently being monitored out of 138 (81.9%) benefited from SMSPS interventions. The teams supported 213 caregivers and 82 people living with HIV, monitored 13 recoveries out of 19 in the community (68.4%), and announced 73 results, including 19 positive ones.
- In **Tshopo**, the teams provided psychological support to 4 confirmed cases at the CTE Météo, carried out psychoeducation for 9 suspects and 108 people and announced 3 results, including 2 positive ones from the previous day; one recovered person was reintegrated into his community.

- **In Haut-Uélé**, 1 new confirmed case, 20 confirmed cases under follow-up, 8 new suspects and 5 suspects under follow-up benefited from SMSPS interventions in Isiro, Boma Mangbetu and Wamba; the teams supported 58 accompanying persons and 46 PPLs and announced 15 results, including 1 positive (data from Pawa not transmitted).

2.9.2. Challenges

- **In Ituri**, only 13 out of 28 affected health zones are covered; urgently strengthen the capacities of psychologists and psychosocial agents for interventions around cases in Mandima.
- **In Haut-Uélé**, difficulties in the mobility of APS in Wamba and Boma Mangbetu, insufficient nutritional kits for weaned infants and poor data reporting in Pawa and Boma Mangbetu.

2.10. Logistics

2.10.1. Main actions

- **In Tshopo**, monitoring of the construction of the CTE in Bafwasende, reception of 11 tonnes of medicines (PHATKIN) in Bafwasende and laboratory inputs (WHO), support for the mobility of pillars and provision of PCI inputs to CUKIS.
- **In Bas-Uélé**, reception of IPC inputs offered by FHI 360, support for the deployment of surveillance teams in Ganga and Viadana, continuation of the rehabilitation of the DPS (WHO) offices and the construction of the transit center of the HGR in Buta.

2.11. Security

2.11.1. Main actions

- **In North Kivu**, the Risk Mitigation and Security pillar carried out 6 security coverage activities, including 100% support for EDS teams for the burial of 3 confirmed cases at the Bundji cemetery (Beni) and for 3 swabs in the Katwa health zone.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

- **In Ituri**, 80 staff members (20 women and 60 men) were briefed on the PSEA and 80 response workers were sensitized in the health zones of Bunia, Rwampara, Aru and Rimba; 831 people (622 women and 209 men) were sensitized in the community with the support of WHO and Mercy Corps.
- **In Haut-Uélé**, 492 people, including 452 students, were sensitized on the PSEAH and the reporting mechanism (495555/151) in Isiro, Wamba and Boma Mangbetu; 8 staff members were briefed, 5 of whom signed the code of good conduct.

2.12.2. Challenges

At the national level, only 2 out of 7 DPS reported their PSEA activities (28.6%) and CBCMs remain largely ineffective.

KEY MESSAGES

- Active and sustained community transmission continues to be observed in the provinces of Ituri, North Kivu, Haut-Uélé, while the province of South Ubangi has not reported any confirmed cases in the last 24 hours.
- Progress to be consolidated has been made in the area of epidemiological investigations: All (100%) suspected cases were investigated within 24 hours of notification. However, the contact tracing rate is higher than the previous day (78.7% vs. 73.5%).
- Strengthen surveillance, care, infection prevention and control (IPC), logistics, and community engagement, prioritizing increased capacity at Ebola Treatment Centers (ETCs)/Treatment Centers (TCs) in North Kivu, rapid transfer of suspected cases, and isolation of confirmed cases. **In Mangobo (Tshopo)**, two confirmed cases who escaped from the ETC remain reported at home.
- Strengthen preparedness and response to Ebola Virus Disease (EVD) in the river corridor and surrounding areas, particularly in South Ubangi, through active case finding, contact tracing, and ensuring the availability of supplies. Continue cross-border surveillance with South Sudan.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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